

NanoGT Match Report: X-linked retinoschisis

Disease: X-linked retinoschisis (ORPHA:792)

Primary gene: RS1

Gene CDS: 672 bp

Inheritance: X-linked recessive

Target tissues: retina

Top 5 GT Precedent Matches

Rank	Program	Vector	Score	Confidence	Approval
1	Luxturna	AAV2	7.5/10	[GREEN] High	approved
2	CPCB-RPE1	AAV8	7.3/10	[YELLOW] Medium	phase2/3
3	Hemgenix	AAV5	7.3/10	[YELLOW] Medium	approved
4	Roctavian	AAV5	7.3/10	[YELLOW] Medium	approved
5	Libmeldy	LV	7.1/10	[YELLOW] Medium	approved

Match #1: Luxturna

Precedent disease: Leber congenital amaurosis type 2

Vector: AAV2

Tissue target: retina/RPE

Composite score: 7.5 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	2.00	2.0	Vector naturally reaches disease target tissue
Protein class	1.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	2.00	2.0	Same or related biological pathway
Inheritance compatibility	0.70	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	0.50	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?

Dimension	Score	Max	What it measures
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	1.00	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	0.80	1.0	Established delivery route to target tissue
TOTAL (normalised)	7.50	10.0	Raw sum / 18×10

Rationale

- Gene CDS 672bp / cargo 4700bp (14% utilized)
- Vector tropism plus precedent target match: retina
- Partial match: extracellular/secreted component present
- LOF inheritance — compatible for gene replacement
- Pathway match: retinal_visual_cycle
- Approval status: approved
- Vector immunogenicity (AAV2): very high (~55%) — majority of patients may be ineligible; major trial design challenge
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted protein — systemic cross-correction via bloodstream; all cells can benefit from a minority of transduced cells
- Immune privilege: highest immune privilege — blood-retinal barrier + FasL + TGF- β 2; minimal T-cell clearance risk
- Promoter availability: VMD2, RPGR, GRK1, CRX, IRBP — multiple validated retinal promoters used in Luxturna, GS010, CPCB-RPE1
- Route of administration: Subretinal/intravitreal injection — specialist ophthalmic procedure; established in Luxturna and GS010

Match #2: CPCB-RPE1

Precedent disease: Achromatopsia

Vector: AAV8

Tissue target: retina/photoreceptor

Composite score: 7.3 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	1.50	2.0	Vector naturally reaches disease target tissue

Dimension	Score	Max	What it measures
Protein class	1.00	2.0	Same se-creted/lysosomal/membrane/intracellular class
Pathway similarity	2.00	2.0	Same or related biological pathway
Inheritance compatibility	0.70	1.0	AR/XL loss-of-function pattern match
Approval precedent	0.70	1.0	Regulatory approval / trial stage
Immunogenicity	1.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	1.00	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	0.80	1.0	Established delivery route to target tissue
TOTAL (normalised)	7.33	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 672bp / cargo 4700bp (14% utilized)
- Precedent target match: retina
- Partial match: extracellular/secreted component present
- LOF inheritance — compatible for gene replacement
- Pathway match: retinal phototransduction
- Approval status: phase2/3
- Vector immunogenicity (AAV8): high (~30%) — substantial patient exclusion expected; immunodepletion protocols may be needed
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted protein — systemic cross-correction via bloodstream; all cells can benefit from a minority of transduced cells
- Immune privilege: highest immune privilege — blood-retinal barrier + FasL + TGF-β2; minimal T-cell clearance risk
- Promoter availability: VMD2, RPGR, GRK1, CRX, IRBP — multiple validated retinal promoters used in Luxturna, GS010, CPCB-RPE1
- Route of administration: Subretinal/intravitreal injection — specialist ophthalmic procedure; established in Luxturna and GS010

Match #3: Hemgenix

Precedent disease: Hemophilia B

Vector: AAV5

Tissue target: liver

Composite score: 7.3 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	0.30	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	0.50	2.0	Same or related biological pathway
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	1.00	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	0.80	1.0	Established delivery route to target tissue
TOTAL (normalised)	7.28	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 672bp / cargo 4700bp (14% utilized)
- No tissue overlap (disease: ['retina'], vector: ['liver', 'lung', 'CNS'])
- Both secreted proteins — systemic delivery viable
- Inheritance match (X-linked recessive <-> XL)
- Different pathway (retinal_phototransduction vs coagulation)
- Approval status: approved
- Vector immunogenicity (AAV5): low (~9%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted protein — systemic cross-correction via bloodstream; all cells can benefit from a minority of transduced cells
- Immune privilege: highest immune privilege — blood-retinal barrier + FasL + TGF-β2; minimal T-cell clearance risk

- Promoter availability: VMD2, RPGR, GRK1, CRX, IRBP — multiple validated retinal promoters used in Luxturna, GS010, CPCB-RPE1
- Route of administration: Subretinal/intravitreal injection — specialist ophthalmic procedure; established in Luxturna and GS010

Match #4: Roctavian

Precedent disease: Hemophilia A

Vector: AAV5

Tissue target: liver

Composite score: 7.3 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	0.30	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	0.50	2.0	Same or related biological pathway
Inheritance compatibility	1.00	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	1.00	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	0.80	1.0	Established delivery route to target tissue
TOTAL (normalised)	7.28	10.0	Raw sum / 18 × 10

Rationale

- Gene CDS 672bp / cargo 4700bp (14% utilized)
- No tissue overlap (disease: ['retina'], vector: ['liver', 'lung', 'CNS'])
- Both secreted proteins — systemic delivery viable
- Inheritance match (X-linked recessive <-> XL)
- Different pathway (retinal_phototransduction vs coagulation)

- Approval status: approved
- Vector immunogenicity (AAV5): low (~9%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted protein — systemic cross-correction via bloodstream; all cells can benefit from a minority of transduced cells
- Immune privilege: highest immune privilege — blood-retinal barrier + FasL + TGF- β 2; minimal T-cell clearance risk
- Promoter availability: VMD2, RPGR, GRK1, CRX, IRBP — multiple validated retinal promoters used in Luxturna, GS010, CPCB-RPE1
- Route of administration: Subretinal/intravitreal injection — specialist ophthalmic procedure; established in Luxturna and GS010

Match #5: Libmeldy

Precedent disease: Metachromatic leukodystrophy

Vector: LV

Tissue target: hematopoietic/CNS

Composite score: 7.1 / 10

Score Breakdown

Dimension	Score	Max	What it measures
Packaging fit	2.00	2.0	Gene CDS size vs vector cargo capacity
Tissue tropism	0.30	2.0	Vector naturally reaches disease target tissue
Protein class	2.00	2.0	Same secreted/lysosomal/membrane/intracellular class
Pathway similarity	0.50	2.0	Same or related biological pathway
Inheritance compatibility	0.70	1.0	AR/XL loss-of-function pattern match
Approval precedent	1.00	1.0	Regulatory approval / trial stage
Immunogenicity	2.00	2.0	Pre-existing NAb seroprevalence for this vector
Therapeutic window	0.50	2.0	Can GT be given before irreversible damage?
Cross-correction	1.00	1.0	Can transduced cells rescue untransduced neighbours?
Immune privilege	1.00	1.0	Immunological protection of target tissue
Promoter availability	1.00	1.0	Validated tissue-specific promoters exist
Route of administration	0.80	1.0	Established delivery route to target tissue

Dimension	Score	Max	What it measures
TOTAL (normalised)	7.11	10.0	Raw sum / 18×10

Rationale

- Gene CDS 672bp / cargo 8000bp (8% utilized)
- No tissue overlap (disease: ['retina'], vector: ['hematopoietic'])
- Both secreted proteins — systemic delivery viable
- LOF inheritance — compatible for gene replacement
- Different pathway (retinal_phototransduction vs leukodystrophy)
- Approval status: approved
- Vector immunogenicity (LV): low (~2%) — most patients eligible; minimal screening burden
- Very narrow therapeutic window — congenital or rapidly fatal early onset; in utero or immediate neonatal GT required; substantially increases trial complexity
- Secreted protein — systemic cross-correction via bloodstream; all cells can benefit from a minority of transduced cells
- Immune privilege: highest immune privilege — blood-retinal barrier + FasL + TGF- β 2; minimal T-cell clearance risk
- Promoter availability: VMD2, RPGR, GRK1, CRX, IRBP — multiple validated retinal promoters used in Luxturna, GS010, CPCB-RPE1
- Route of administration: Subretinal/intravitreal injection — specialist ophthalmic procedure; established in Luxturna and GS010